CHANNEL PRICING · AUDIT WORKSHEET

Same Drug. Three Channels. Three Prices.

A worksheet for auditing per-channel net cost on your top 10 drugs by spend. The retail, mail-order, and specialty channels rarely match — and the spread is the design choice your PBM made for you.

01 Before You Audit: Data to Pull from Your PBM

Request a 12-month claims extract by drug and by channel for your top 10 drugs by total spend. The single diagnostic data request is per-channel net cost; if the PBM cannot produce it, that itself is a finding worth bringing to your next renewal review.

- ☐ Top 10 drugs by total annual plan spend (12-month)
- ☐ Channel utilization split per drug (% of fills at each channel)
- ☐ Pricing methodology by channel (AWP discount, dispensing fee, exclusions)
- ☐ Net cost per script for each, broken out by channel (retail / mail / specialty)
- ☐ Currently active formulary tier per channel
- ☐ Specialty channel guarantees that are excluded from the contract pricing performance

02 The Worksheet

Fill in net cost per script for each channel where the drug fills. Calculate variance as (highest channel net cost – lowest channel net cost) ÷ lowest. Mark the variance band per the page-2 reference.

DRUG	ANNUAL SPEND	RETAIL NET COST	MAIL NET COST	SPECIALTY NET COST	VARIANCE BAND
1.	_____	_____	_____	_____	○ ○ ○
2.	_____	_____	_____	_____	○ ○ ○
3.	_____	_____	_____	_____	○ ○ ○
4.	_____	_____	_____	_____	○ ○ ○
5.	_____	_____	_____	_____	○ ○ ○
6.	_____	_____	_____	_____	○ ○ ○
7.	_____	_____	_____	_____	○ ○ ○
8.	_____	_____	_____	_____	○ ○ ○
9.	_____	_____	_____	_____	○ ○ ○
10.	_____	_____	_____	_____	○ ○ ○

Illustrative worksheet for educational purposes. Specific variance band thresholds should be calibrated to your plan size, drug mix, and channel utilization. The audit identifies channels where the price is materially out of line with the others, not whether channel routing should change.

03 Variance Bands · How to Read Your Worksheet

Once each row of the worksheet is filled in, calculate variance per drug as (highest channel net cost – lowest channel net cost) ÷ lowest. Mark the variance band on each row. The pattern across the top 10 is what matters; a single high-variance drug may be drug-specific, but multiple high-variance drugs is a contract design conversation.

TIGHT VARIANCE · UNDER 15%

Channel pricing is well-aligned across your top drugs.

The discounts, dispensing fees, and methodology are producing similar per-unit prices regardless of channel. This is the cleanest contract structure to inherit. Maintain by requiring per-channel net cost as a standing quarterly deliverable.

MODERATE VARIANCE · 15-30%

Channel-level differences are real but reasonable.

Some spread is normal — mail order's deeper discount and specialty's separate methodology will move per-unit prices. The investigation is whether the spread tracks your plan's interests (i.e., the lower-cost channel is also the higher-utilization channel). If utilization concentrates in the higher-cost channel, that is a routing conversation.

WIDE VARIANCE · OVER 30%

Channel design is materially out of line with your plan.

Spreads above 30% on multiple top drugs typically signal one of three things: specialty pricing methodology with weak guarantees, mail-order routing that captures the low-cost channel for the PBM rather than the plan, or a formulary tier that pushes the plan into the highest-cost channel by default. Each requires a different intervention; the audit names which.

VARIANCE REPORTING GAP

PBM cannot produce per-channel net cost.

If the PBM declines or cannot produce the per-channel net cost data, that itself is a finding. The contract did not require it; the next renewal redline should. Document the response (in writing if possible), date it, and bring it to the renewal conversation as a contract-language ask.

04 Action Plan and Contract Asks

Findings without action are observations. Build from the worksheet. Channel pricing gaps live in the contract; contract gaps close at renewal. The asks below are paste-ready language for your next renewal redline conversation.

§ **Open the channel pricing conversation pre-renewal**
Bring the completed worksheet to the next standing PBM meeting, not the renewal meeting. Channel pricing is rarely on the standard agenda; surfacing it before renewal lets the conversation happen on your timeline rather than the PBM's negotiation timeline.

§ **Push for net cost transparency per channel as standing deliverable**
Per-channel net cost on top 10 drugs becomes a quarterly report, not a custom request. Include the data format, the lookback window, and the delivery cadence in the contract amendment. The standing deliverable closes the visibility gap permanently.

§ **Audit specialty pricing methodology specifically**
Specialty channel pricing typically uses a different methodology than retail and mail, often with separate guarantees that are excluded from the overall contract pricing performance language. Most PBM contracts allow this gap. The redline is to bring specialty pricing under the same performance language with the same remediation triggers.

§ **Add channel-pricing performance guarantees at next renewal**

Specific contract clauses to add: GER (Generic Effective Rate) guarantees by channel, brand discount guarantees by channel, dispensing-fee schedules disclosed by channel, and remediation if any channel-level guarantee misses. Combine with quarterly per-channel reporting as the verification mechanism.

§ **Schedule channel pricing review on the benefits committee calendar**

Quarterly cadence; document each quarter's variance bands and the actions taken since the prior quarter. This builds the fiduciary documentation trail (see Week 19 framework) at the same time it protects the plan.